Responsive Caregiving and Attachment Interventions

Randomized sensitivity-training interventions reliably raise caregiver sensitivity and, to a lesser degree, infant attachment security.

ATE-TO-HIGH-SENSITIVITY; MODERATE-ATTACHMENT | STATUS: DRAFT | DOMAIN: ATTACHMENT, PARENTING, SOCIAL | REPLICA

WHAT WE KNOW (Narrowest Supported Conclusion)

Randomized interventions that train parental sensitivity produce a medium-sized increase in sensitive caregiving and a smaller but real increase in infant attachment security; short, focused, behaviorally-oriented interventions outperform longer or more diffuse ones.

EVIDENCE AND CAUSAL ASSESSMENT

- Bakermans-Kranenburg, van IJzendoorn & Juffer (2003) meta-analysis: 70 studies, 88 intervention effects (sensitivity N=7,636; attachment N=1,503). Randomized interventions: d=0.33 for sensitivity, d=0.20 for attachment security.

- Earlier van IJzendoorn et al. (1995) meta-analysis (16 studies, N=869) found d=.58 for sensitivity change and d=.17 for attachment security change; short-term preventive interventions outperformed longer/more intensive/therapeutic approaches.

- De Wolff & van IJzendoorn (1997) meta-analysis of parental antecedents of attachment (66 studies, N=4,176) established sensitivity as a moderate, non-exclusive predictor of attachment security.

EFFECT SIZE AND PERSISTENCE

Sensitivity change: d~0.33-0.58 (medium). Attachment security change: d~0.17-0.20 (small). These are intervention (efficacy) effects from mostly short-term RCTs; long-run persistence into middle childhood and beyond is not established by this literature pass.

BIAS, HETEROGENEITY, AND CONFOUNDING

GI attenuation: n/a (randomized sensitivity intervention trials, not an observational population phenotype).

Moderator analyses consistently show shorter, more behaviorally focused interventions outperform longer/more clinical ones. Effectiveness in multi-problem/high-risk families appears comparable to lower-risk families, suggesting some scalability, but effectiveness trials and cross-cultural replication outside Western/high-income settings are less represented.

WHAT THIS DOES NOT MEAN (Strict Boundary Conditions)

Does not establish that insensitive parenting is a dominant determinant of child outcomes, nor that all attachment insecurity is parent-caused.

WHAT ACTION IS REASONABLY JUSTIFIED

Short, focused, behaviorally-oriented sensitivity-training programs are a reasonably well-supported intervention target for services aiming to support caregiving quality, particularly in higher-risk families; this does not justify blanket judgments about individual parenting style.

VERIFIED SOURCES & DOI IDENTIFIERS

- Bakermans-Kranenburg, van IJzendoorn & Juffer (2003), Psychological Bulletin
- van IJzendoorn, Juffer & Duyvesteyn (1995), J Child Psychol Psychiatry
- De Wolff & van IJzendoorn (1997), Child Development